

Recommended, reimbursed — and still not completed.

A partnership brief on closing the gap between the preventive care that is recommended and the preventive care that actually gets finished — in the safety-net settings where the gap is widest.

THE PITCH IN 30 SECONDS

PROPOSED · IN DEVELOPMENT

PROBLEM	U.S. colorectal-cancer screening sits at 63.5% against a 72.8% target — and just ~44% in Federally Qualified Health Centers. The services work; completion is what fails.
SOLUTION	A proposed disease-agnostic completion engine : it finds who is overdue, removes friction, and drives follow-through — with an explainable, fairness-audited dashboard.
EVIDENCE	Every lever is already recommended by the Community Preventive Services Task Force (mailed FIT +16.1pp, reminders +15.3pp, navigation +13.6pp). Honest caveat: real-world gains depend on implementation fidelity — STOP CRC averaged ~3.4pp — which this design exists to protect.
COST	~\$90 per additional person screened.
THE ASK	One clinic to run a pre-registered 12-month pilot ; partners to scale it.

THE PROBLEM — ADOPTION, NOT SCIENCE

~8%
of U.S. adults 35+ receive all 15 high-priority preventive services (Borsky, *Health Affairs* 2018).

63.5% → 72.8%
CRC screening vs. the Healthy People 2030 target — barely off baseline (2023).

~44%
CRC screening in Federally Qualified Health Centers (2018 UDS) — the gap concentrates here.

WHICH ONE ARE YOU? — DIFFERENT PARTNERS, DIFFERENT VALUE, SAME ENGINE

HEALTH CENTERS / FQHCS

Improve a measure you already report

Host the pilot — raise a UDS/HEDIS measure you already report, inside your existing staffing, with the fidelity discipline that makes outreach reliably land.

FUNDERS / PARTNERS

Fund a cost-effective, scalable model

A disease-agnostic engine at ~\$90 per additional person screened, with an equity-stratified design and a path from one site to a network.

RESEARCHERS

Co-design a publishable pilot

A pre-registered stepped-wedge / RE-AIM evaluation with a protocol-first publication — rigor built in, co-authorship on the table.

SIX PILLARS, ONE COMPLETION LOOP

- Service redesign & friction reduction
 - Agentic & conversational outreach
- Interoperable data infrastructure
 - Algorithmic equity & governance
- Predictive AI, explainable by design
 - Implementation enablement

Identify → Reduce friction → Communicate (literacy-tailored) → Escalate (community-health-worker outreach) → Measure, incl. diagnostic follow-up after a positive result.

THE DEMONSTRATION

A proposed 12-month colorectal-cancer completion pilot.

One Southeast Michigan FQHC below the UDS benchmark, with a FHIR-capable EHR and a quality-improvement sponsor. Evaluated with RE-AIM against the site's own baseline, pre-registering a benchmarked target above the ~3.4-point STOP CRC average, with a comparison clinic where feasible, a named post-grant sustainability path, and guaranteed diagnostic follow-up. Disease-agnostic by design — the same loop extends to cervical and breast screening, hypertension, and immunizations.